



Armada Kalamunda Group

Alcohol and Other Drugs – Patient Management Guideline

1. Purpose

The purpose of these guidelines is to provide staff caring for patients who may be affected by alcohol and other drugs (AOD) within Armada Kalamunda Group (AKG) and in accordance with the WA Health [Smoke Free Policy MP 0158/21](#) and East Metropolitan Health Service (EMHS) [Alcohol and Prohibited Substances Policy EMHS 112](#).

Information is provided in each section specific to alcohol or specific drug group including initial considerations, signs and symptoms of intoxication and withdrawal, assessment and management

2. Applicability

This guideline applies to all services and staff working across the Armada Kalamunda Group.

3. Guidelines

3.1 Key practice points:

- Early detection of withdrawal and preventing the risks associated with withdrawal are the key components for effective and safe management. All patients at risk of experiencing alcohol withdrawal are to be commenced on an Alcohol Withdrawal Chart.
- Consideration should be given to prescribing thiamine to reduce the risk Wernicke's Encephalopathy (WE) or Korsakoffs Syndrome (KS).
- Oral thiamine is ineffective in moderate to severe thiamine deficiency and is not to be used in the Emergency Department (ED).
- If there is reasonable concern that a mental health inpatient has consumed alcohol and / or illegal drugs, the in-patient can be requested to submit to a breath and / or blood test. All breathalyser testing must be performed by trained staff, with relevant medical staff consulted in the decision to test the patient.
- Alcohol, tobacco and other drugs screening should occur in pre-admission clinic where possible.
- All patients are to be advised on admission that the use of alcohol, tobacco and illicit drugs / substances will not be permitted whilst they are in the hospital.
- Pregnancy - Consider specialist advice from obstetric physician.
- The WANDAS may be contacted for specialist advice for pregnant patients.
- Psychiatric illness - Consider possibility of exacerbation of psychological symptoms. The admission process includes a comprehensive drug and alcohol assessment.
- Elderly - Consider the aspect of longer periods of use and dependence which may complicate treatment and compromise recovery goals.

- Legal competence may be a consideration in relation to alcohol, tobacco or other drug usage and consent to treatment.
- The Medicines and Poisons Act 2014 requires authorised medical practitioners / nurse practitioners to report to the Department of Health (DoH) Medicines and Poisons Regulation Branch within 48 hours, when they have reason to believe a person has acquired, as a result of repeated administration of drugs of addiction, an overwhelming desire for the continued administration of a drug of addiction. Reports of dependence will be made using the approved 'Reporting a Drug Dependent Person' or 'Reporting an Oversupplied Person' form.
- Prior written authorization must be received from the DoH before prescribing Schedule 8 discharge medicines to a person who has been recorded as a Drug Dependent Person.
- The Health Regulations 2010 requires a psychiatrist to notify the DoH Medicines and Poisons Regulation Branch within 72 hours of making a diagnosis of stimulant induced psychosis is made by completing the 'Notification Stimulant Induced Psychosis' form.
- Cultural Issues: –
 - Consider the availability of appropriate liaison staff,
 - use of interpreters when necessary,
 - other cultural aspects that may affect setting, expectations, family involvement, follow up and other issues.

4. Assessment

4.1 Patient history taking will include:

- which substance(s) are or have been used
- blood borne viruses (BBV)
- sexually transmitted infections (STI)
- pregnancy
- seizure history
- physical co-morbidities
- mental health co-morbidities
- social history including dependents, housing
- cognitive assessment.

Physical examination will include:

- general/focussed physical examination
- evidence of intoxication
- evidence of withdrawal
- evidence of complications from substance use.

Laboratory and other testing will include:

- full blood count
- serum electrolytes including potassium, magnesium and phosphate
- Glucose
- Creatinine
- liver function tests

- Lipase
- blood alcohol level
- urine drug testing
- Beta Human Chorionic Gonadotropin (HCG).

Readiness to Change

- Stages of Change Model (Prochaska & Di Clemente)
- Pre-contemplation, contemplation, preparation, action, maintenance
- Match the intervention to the person's stage of readiness for change
- Refer to [Prochaska and Di Clemente's Stages of Change Model](#) to guide patient management plan.

Initial Management

- Identify and manage immediate harms and risks (e.g. overdose, suicidal ideation, behavioural disturbance, severe withdrawal)
- Discuss referral (inpatient and community, as applicable)

Referral to specialist services

- Referrals to specialist mental health/AoD Consultation Liaison Services will be initiated when clinically indicated.
- ED patient can be referred by phone or direct consult – An e-referral completed is to be complete
 - Inpatient > Psychiatry.
- Inpatient wards referrals are via e-Referral and phone call
- Afterhours information via Mental Health Commission website for resources or contact Clinical Advisory Service (CAS 24/7) directly – 9442 5042 – health professional contact number only.

5. Alcohol

5.1 Alcohol intoxication

- Alcohol is a central nervous system depressant / sedative drug.
- Even in small doses, alcohol significantly impedes motor and cognitive function depending on dose and individual susceptibility.
- Intoxication can be exacerbated by the intake of other medications refer to [Appendix 3](#)
- The AHS **Alcohol Withdrawal Chart (CIWA-Ar) AKMR 147B** must be commenced for the management of symptoms of alcohol withdrawal.

5.2 Signs and symptoms

- Varying degrees of talkativeness, euphoria, disinhibited and bizarre behaviour.
- Ataxia, uncoordinated motor function.
- Pressured or slurred speech.
- Double vision, impaired memory, impaired judgment.
- Hypothermia, hypotension.
- Nausea, vomiting, abdominal pain.

- Dysrhythmia, seizures.
- Sleepiness, poor response to external stimuli.
- Respiratory depression, coma, death.

5.3 Management of alcohol intoxication

- Nurse the patient in an area:
- Easily visualised by staff.
- Ongoing assessment of signs of nausea and vomiting should occur.
- A reduced gag reflex may result in a risk of airway occlusion and aspiration of vomitus.
- Clinical management will be based on the clinical condition of the patient and the escalation protocols on the Adult Observation and Response Chart (AORC) and CIWA-Are and in consultation with the Medical Officer (MO)/Senior Clinician and the AOD service
- Access to resuscitation equipment, oxygen and suction.
- Position the patient to reduce risk of aspiration.
- Minimum requirements will be based on the clinical condition of the patient and in consultation with the Medical Officer / senior clinician.
- The level of intoxication will be assessed by breath alcohol reading or blood alcohol concentration test. This can be done by a breath alcohol reading or blood alcohol concentration test (trained staff only to perform).
- Report the patient's mental state and concerns Discuss with the MO / seniors clinicians and consider assessment tools, to be used by trained staff.
- Where possible reducing environmental stimuli e.g. provide single room, dim lights.
- Medical review must occur if there is any identified change to the patient's neurological observations and assessments including Glasgow Coma Scale, speech, eye movements and pupil abnormalities [AKG Full Neurological Observations Procedure](#)
- Administer medications as prescribed on the patient medication chart.
- Encourage diet and fluids. Administer intravenous fluids as prescribed.
- Consider monitoring urine output if there is evidence of potential renal impairment.

5.4 Alcohol withdrawal

Initial considerations:

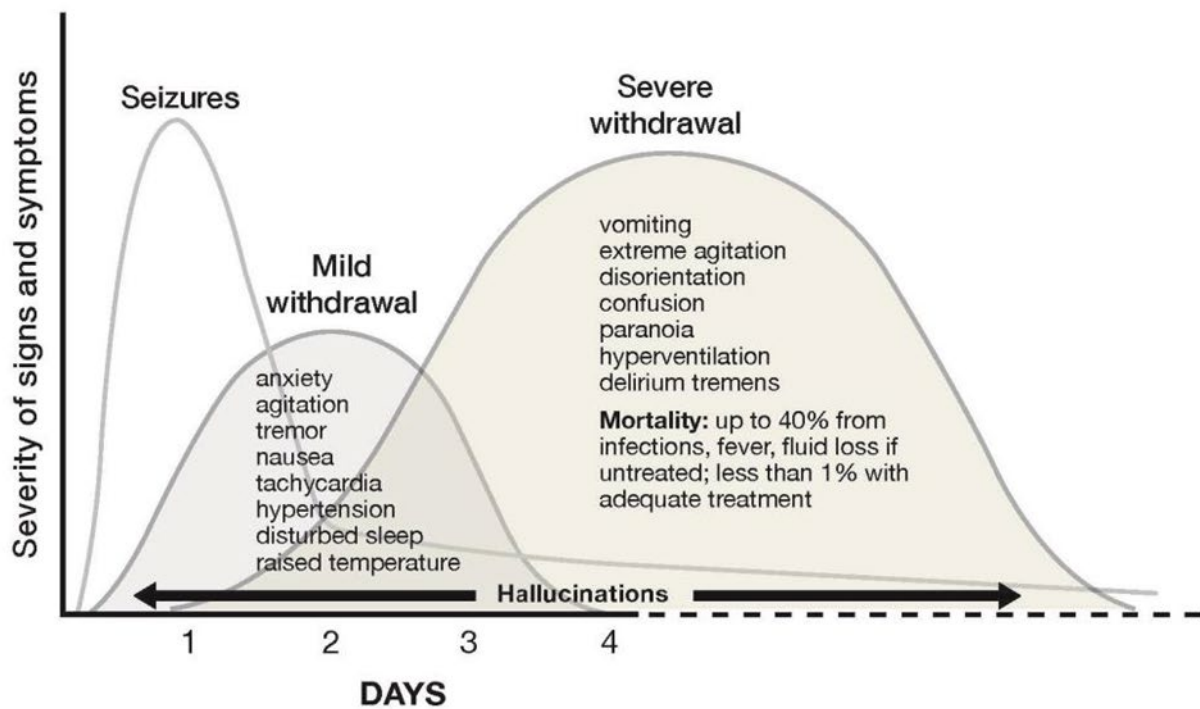
- The signs and symptoms of alcohol withdrawal may be confused with other problems common in critical care patients such as electrolyte imbalances, pain, and infection.
- The risk of major withdrawal is greater in people with acute medical conditions (e.g. trauma or sepsis).
- Onset of alcohol withdrawal is usually 6-24 hours after the last drink and can occur before alcohol level is <0.01% or breath alcohol is 0.000%.
- In people with severe alcohol dependence withdrawal can occur when the blood alcohol level is decreasing, even if the patient is still intoxicated.
- Seizures affect 5% of patients, occurring early usually 7-24 hours after the last drink.
- Seizures are tonic clonic in type i.e. generalised not focal and usually (though not always) occur as a single seizure.

Table 1: Signs and symptoms of Alcohol Withdrawal Classification

	Autonomic Hyperactivity	Gastrointestinal features	Cognitive and Perceptual Changes
Uncomplicated withdrawal	Sweating Tachycardia Hypertension Tremor Fever (generally < 38OC)	Anorexia Nausea Vomiting Dyspepsia Diarrhoea	Poor concentration Anxiety Psychomotor agitation Disturbed sleep Vivid dreams
Severe withdrawal Complications	Dehydration and electrolyte disturbance		Seizures Hallucinations or perceptual disturbances (visual, tactile, auditory) Delirium

Table 2: Progress of Alcohol Withdrawal Syndrome

Progress of alcohol withdrawal syndrome



Adapted from Frank L., Peard J. New concepts in drug withdrawal: a resource handbook: ©1995 State of Victoria. Reproduced with permission.

Table 3: Predictors of Alcohol Withdrawal Severity

Current Drinking Patterns	The severity of withdrawal is only moderately predicted by amounts of alcohol consumed. Chronic heavy alcohol consumption (15 standard drinks per day) is associated with greater withdrawal severity than lower levels of consumption. A predictor of increased alcohol withdrawal severity is the onset of withdrawal symptoms (such as tremor, nausea, anxiety) upon waking that are normally relieved by early morning drinking. Individuals with heavy but irregular alcohol consumption (2-3 days per week), sometimes referred to as binge drinking, generally do not experience severe withdrawal.
Past Withdrawal Experience	Patients with a history of severe alcohol withdrawal syndrome (such as severe anxiety, seizures, delirium, hallucinations) are more likely to experience similar complications in future withdrawal episodes.
Poly Substance Use	Patients with a heavy or regular use of other substances (such as benzodiazepines, stimulants, opioids) may experience more severe withdrawal features. In particular, withdrawal from both alcohol and benzodiazepines may increase the risk of withdrawal complications. Refer to Appendix 9: Drugs with the potential to interact with alcohol
Medical or Psychiatric Comorbidity	Patients with concomitant medical conditions (such as sepsis, epilepsy, severe hepatic disease, head injury, pain, and nutritional depletion) or psychiatric conditions (such as anxiety, psychosis or depression) could be more likely to experience severe withdrawal complications.

5.5 Acute alcohol delirium

Acute alcohol withdrawal delirium can be difficult to distinguish from other forms of delirium and in the absence of comprehensive history, alcohol withdrawal may go untreated.

5.6 Aims of alcohol withdrawal management

Effective withdrawal management is through symptom identification and supportive care and / or medication. It aims to prevent:

- Wernicke's encephalopathy
- Progress to severe withdrawal.
- Injury to self and others due to mental state.
- Dehydration.
- Electrolyte and nutritional disorders.
- Potential for seizures.
- Complications due to underlying illness. Which include: Liver disease.
- Pneumonia.
- Gastrointestinal (GI) bleeding.
- Hypoglycaemia.
- Electrolyte imbalance.

5.7 Pharmaceutical interventions

- Benzodiazepines (diazepam) is the treatment of choice for alcohol withdrawal due to good oral absorption, rapid onset and prolonged duration of effects.
- Benzodiazepines should not be abruptly withdrawn but slowly reduced in a strict regime. Consider specialist advice re titrating regime and risk reduction strategies such as staged supply.
- Lorazepam is preferred where there is concern over prolonged sedation such as the elderly, patients with recent head injury, liver failure or respiratory failure
- In patients with decompensated cirrhosis and alcohol dependence lorazepam will be used instead of diazepam due to shorter acting duration and minimization of accumulation 1 milligram of Lorazepam per oral is equivalent to 5 milligrams of diazepam per oral
- Thiamine reduces the risk of precipitating Wernicke's encephalopathy, and / or Korsakoff's syndrome.
- Multivitamin and folate supplements.
- All pharmaceutical intervention must be administered as per the multidisciplinary management plan.
- The patient must be observed and documentation of this must occur as per policy and protocols.

Table 4: Treatment regime

Treatment regime	Drug dose	Setting	Patient
Loading dose therapy	Diazepam 10-20mg orally every 2 hours until 60 – 80mg given or lightly sedated. Maximum dose 120mg Diazepam in 24 hours. Reducing over 3-5 days with fixed dosing regimen plus CIWA-Ar	Emergency department/Intensive care areas	Severe withdrawal CIWA-Ar >20 High risk of complex withdrawal e.g. delirium, seizure, hallucinations. History withdrawal seizures.
Fixed schedule therapy	Day 1: 20mg diazepam QID plus CIWA-Ar for additional prn if indicated. Reducing doses over 4-5 days	General wards Mental Health areas	Moderate to severe alcohol withdrawal in medical setting (CIWA-Ar > 9). History of significant withdrawal symptoms, withdrawing from alcohol and other substances. Significant medical or psychiatric co - morbidity.
Symptom triggered therapy	PRN Diazepam to be prescribed and given as per CIWA-Ar if required.	General wards Mental Health areas	Mild to moderate alcohol withdrawal (CIWA-Ar 0-8) No history of alcohol withdrawal

Treatment regime	Drug dose	Setting	Patient
			complications. No significant co - morbidity

5.8 Thiamine replacement therapy

- Thiamine deficiency is common in patients with chronic misuse of alcohol and may lead to potentially life threatening conditions such as WE.
- There is little evidence to support what the optimal regime of thiamine supplementation is to prevent neurotoxicity.
- Orally administered thiamine has poor bioavailability with a maximum of 4.5 mg available per 100mg ingested. This is further reduced by up to 70% in the context of alcohol intoxication and malnutrition.
- Magnesium is required for optimal utilisation of thiamine. Check serum magnesium and replace if below the normal range (0.70-1.10mmol/L).
- Thiamine replacement therapy reduces the risk of precipitating WE and/or Korsakoff's Syndrome Intravenous (IV) thiamine administration is the preferred route, although the intramuscular (IM) route is acceptable if there is no IV access. IM route is only suitable for doses up to 200mg.
- Patients presenting to ED with alcohol intoxication and proven or suspected chronic alcohol misuse should receive:
 - 200mg thiamine as an intermittent IV infusion
 - followed by 200mg daily IV whilst in hospital.
- Patients with known or suspected alcohol withdrawal and a clear or altered mental state from any cause should receive:
 - 200mg thiamine IV eight hourly with dose review in 3 days
 - Patients with severe features of WE e.g. confusion or coma, should be prescribed:
 - 500mg IV eight hourly for at least 3 to 5 days and resolution of symptoms.
 - Additional thiamine treatment should be based on clinical response and should continue until there is no further clinical improvement.
 - Following cessation of IV thiamine chronic drinkers with poor dietary intake and general poor nutritional status should be maintained on oral thiamine at 300mg per day for at least two weeks
 - Oral thiamine supplementation (100 – 300mg PO daily) should be continued indefinitely in an alcohol dependent patient who continues to drink and should be considered on discharge)

5.9 Alcohol withdrawal – The CIWA-Ar

- is validated for alcohol withdrawal only and should not be used to assess withdrawal from any other substance.
- is not to be used as a diagnostic tool, as many other conditions produce similar signs and symptoms.
- can be ceased when the patient has a total score of eight or less on four consecutive occasions once the patient is 48 hours post alcohol cessation.

6. Benzodiazepines

Benzodiazepines are effective anxiolytic hypnotic medications. They are also effective amnestic agents and may have a disinhibitory effect especially at high doses. However, prolonged use is associated with problematic tolerance and dependence

6.1 Initial considerations

- Benzodiazepines are used in clinical practice for sedation and the management of anxiety, seizures, withdrawal states, insomnia and drug associated agitation.
- Persons using high doses may be seeking 'emotional blockade' against previous psychological trauma.
- Benzodiazepines should only be used for short term treatment. Dependence develops relatively quickly, though this varies with each patient. Within 3-12 months of use 10-20% of patients may become dependent and this increases to 20-45% after more than a year of use.
- Tolerance of different effects of benzodiazepines develops at different rates i.e. tolerance of sedative effects begins after two or three days and is significant by two to three weeks.
- Benzodiazepine users may be classified as having 'therapeutic dependence' or as being 'poly-substance users'.

6.2 Therapeutic dependence

- Many people take benzodiazepines prescribed from a single doctor for many years without dose escalation for management of anxiety or insomnia.
- These patients have great difficulty ceasing use because they wish to avoid withdrawal and/or a return of the initial presenting symptoms.
- Risks associated with therapeutic dependence are poorly defined however it is known that long term use in the elderly is associated with an increased rate of cognitive decline and risk of falls. In this patient cohort the risk of continued use needs to be balanced against the distress and benefits of withdrawing.
- Patients in hospital for other reasons may undergo benzodiazepine withdrawal from even low doses of their regular long term benzodiazepines. This can be particularly problematic for the elderly who may develop delirium due to withdrawal.
- Therapeutic dependence is best managed by very slow withdrawal supervised by the patients General Practitioner. Advice on strategies for optimising sleep and controlling anxiety should also be given.

6.3 Screening and assessment

- The critical issue at assessment is to classify patients according to whether they are dependent on therapeutic doses of benzodiazepines or are intermittent or regular high-dose users (usually in the context of multiple drug use).
- History of benzodiazepine use is critical including collateral information from the General Practitioner (GP) and the PSP.
- The patient may be intoxicated on presentation which may impact their ability to provide and receive information.
- Assessment of intoxicated individuals should be limited to ensuring their safety. Caution should be taken when prescribing medications.
- Withdrawal scores alone should not be relied on to guide management.

- When a patient meets the PSP criteria, verbal advice or a patient summary report on the PBS medicines dispensed and the number of prescribers involved during a defined period can be supplied.
- Further information on the PSP is available on the Australian Government Department of Human Services website.

Table 5: Intoxication signs and symptoms

Short term effects	Long term Effects
• Drowsiness and fatigue • Muscle weakness • Impaired memory and cognition • Intoxication from BZD can present as sedation where the individual can be roused but quickly lapses back to the sedated state • Slurred speech and drooling • Poor balance and incoordination • Disinhibition • Paradoxical stimulating effect	• Tolerance • Dependence • Withdrawal syndrome from discontinuation • Emotional blunting • Cognitive impairment • Menstrual irregularities, breast engorgement

- Following oral administration symptoms peak from 15 minutes to 2.5 hours 20 [Level GPP]. Benzodiazepine overdoses are of significant concern, although, rarely life-threatening on their own. Patients with substance use problems often use BZD concurrently with other substances which greatly increase the risk of overdose.

6.4 Withdrawal

- Early detection of withdrawal and the prevention of risks associated with withdrawal are the key components for safe and effective patient management.
- Benzodiazepine withdrawal is usually mild, and symptoms are generally the same as those for which the benzodiazepines were initially prescribed. High dose use is more likely to present with severe symptoms.
- Where low-dose therapy is continued for six weeks or longer, tolerance and symptoms of referral will affect 15-50% of patients
- Withdrawal is more likely in:
 - usage exceeding three months
 - doses higher than therapeutically recommended
 - passive and dependent personality traits
 - sudden cessation of use

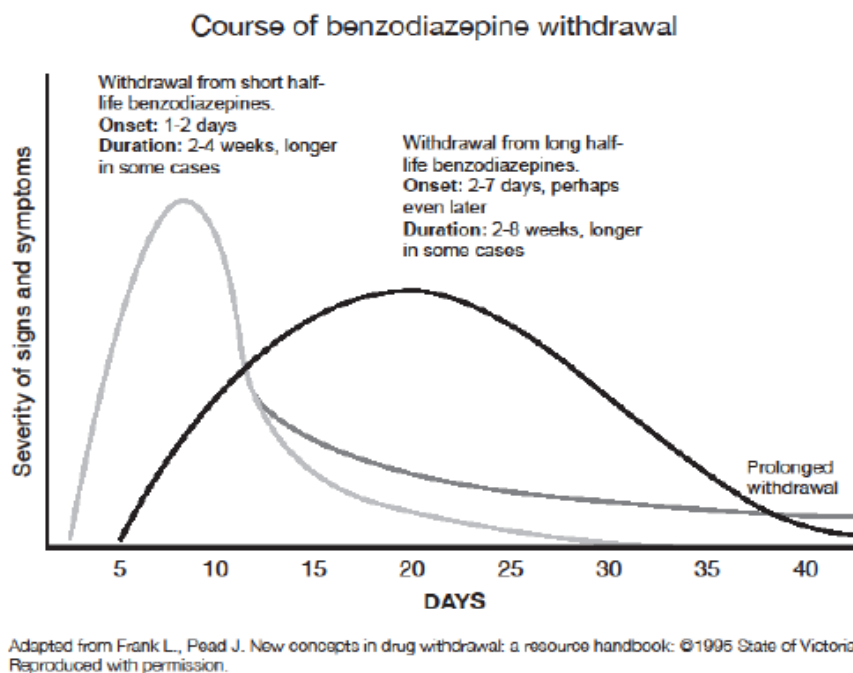
6.5 Withdrawal onset

- Onset occurs two – five days after stopping, reaching a maximum on days seven – ten and usually abating by the end of the second or third week

Table 6: Signs and Symptoms of BZD Withdrawal

Common	Less Common
• Anxiety • Agitation and irritability • Restlessness • Insomnia • Tremors, muscle aches and twitches • Depression • Poor concentration and memory	• Seizures • Delirium • Hallucinations • Depersonalisation • Nausea • Anorexia • Hypersensitivity to sound, light, touch and taste

Table 7: Benzodiazepine withdrawal course



6.6 Withdrawal management

- The aim of withdrawal management is to cease the benzodiazepine use safely without delirium or seizures.
- Wherever possible the community setting is the preferred option for managing withdrawal with:
 - an organised, structured and gradual regimen
 - General Practitioner (GP) involvement
 - linking to community alcohol and drug services to provide counselling and management as required

- Benzodiazepine withdrawal scales are not used at AKG as there is little research to support their validity, reliability and predictive value.

6.7 Inpatient management

- Benzodiazepine use should not be abruptly discontinued even at low doses due to the potential risk of precipitating withdrawal and seizures.
- For patients with therapeutic dependence maintain benzodiazepines at preadmission levels.
- Patients on high doses or those who are poly-drug users should have a reduction plan initiated when medically stable.
- Refer to general nursing care requirements – Section 11
- Pharmaceutical management of inpatient withdrawal
- Where inpatient withdrawal is required refer the patient to the appropriate AOD service contact or the AOD CAS.
- Withdrawal is best managed by stabilising the patient on one long acting benzodiazepine and then progressively reducing the dose.
- Patients taking high doses of benzodiazepines or poly-drug users should be stabilised on a long acting benzodiazepine (preferably diazepam) at a dose of approximately 40% of their regular intake before admission or 80mg/day whichever is the lower dose.
- Different benzodiazepines have different length of action and half-life see below.

Table 8: Comparative oral doses of benzodiazepines

Drugs	Length of action	Half-life	Approximate equivalent dose to 5mg diazepam
Alprazolam	Short	6-12 hours	0.5mg
Bromazepam	Medium	12-24 hours	3mg
Clobazam	Long	> 24 hours	10mg
Clonazepam	Long	> 24 hours	0.25mg
Diazepam	Long	> 24 hours	5mg
Flunitrazepam	Long	> 24 hours	0.5mg
Lorazepam	Medium	12-24 hours	1mg
Nitrazepam	Long	> 24 hours	5mg
Oxazepam	Short	6-12 hours	15mg
Temazepam	Short	6-12 hours	10mg
Triazolam	Short	6-12 hours	0.25mg

- Contact the DACAS when prescribing greater than 40mg of diazepam per day to discuss treatment options as the risk of overdose is higher.
- Reduction and withdrawal should be initiated once the patient is medically stable.

- Dose reductions should not exceed 10-20% per week and be titrated against symptoms. Rates of reduction need to be balanced with co-occurring issues such as anxiety which may be exacerbated during the withdrawal period.
- For patients stabilised on 40-80mg of diazepam daily withdrawal should be at a rate of:
 - at least 5mg per week until 40mg daily dose is achieved then
 - withdrawn at a rate of 2.5 mg per week thereafter e.g. reducing from 80mg diazepam daily at this rate would take nearly six months.
- The maximum rate of withdrawal in a patient stabilised on 40-80mg diazepam daily is:
 - to reduce the daily dose by 10mg at weekly intervals until the patient is on 40mg daily
 - then reduced by 5mg at weekly intervals, reducing from 80mg diazepam daily at this rate would take 12 weeks.
- Adjunct medications may be prescribed including:
- Beta Blockers to control anxiety and autonomic hyperactivity
- Antidepressants for sedative effect (tricyclic antidepressants) or for the treatment of an underlying depressive disorder (serotonin reuptake inhibitors)
- Anticonvulsants may be prescribed for patients at increased risk of seizures i.e. chronic comorbid alcoholism or persistent high dose benzodiazepine users.
- The patient will be provided with clear information advising the aims of withdrawal and informing them they may feel uncomfortable and experience withdrawal symptoms.
- The reduction plan should be continued on discharge and the benzodiazepine dispensed daily with supervision at their local pharmacy.

7. Cannabis

The generic term given to the psychoactive substances found in the marijuana plant Cannabis Sativa. The main active constituent is delta9-tetrahydrocannabinol (THC) and although this is not classed as a hallucinogen it has a similar effect

7.1 Initial considerations

- Cannabis is considered psychotogenic and in vulnerable individuals is associated with new presentations or exacerbation of psychosis during intoxications and occasionally cannabis withdrawal
- Underlying psychiatric illnesses or symptoms, which may have been masked during cannabis use, could emerge during the withdrawal period.
- Cannabis can be taken orally or can be smoked, often with tobacco, and the pharmacokinetics and pharmacodynamics of THC vary by route of exposure.
- Inhaled marijuana:
 - onset of psychoactive effects occurs rapidly with peak effects felt at 15-30 minutes and lasting up to four hours.
 - effects will mirror the plasma THC concentrations.
 - approximately 2-3 mg of inhaled THC is sufficient to produce drug effects in a naïve user.
- Ingested marijuana:

- has a comparatively delayed onset of psychoactive effects that ranges from 30mins to three hours and clinical effects may last up to 12 hours.
- is less potent due to chemical degradation in gastric acid and poorer absorption.
- approximately 5-20mg of ingested THC is sufficient to produce drug effects in a naïve user.
- Patient factors such as weight, metabolism, gender and eating habits will influence how soon and for how long someone will feel intoxicated
- Cannabis contains as many carcinogens as tobacco, deposits a third more tar and is thought to be a risk factor for oropharyngeal and lung cancer and coronary heart disease.

7.2 Intoxication signs and symptoms

- The signs and symptoms of cannabis intoxication include:
 - tachycardia, hypertension and tachypnoea
 - conjunctival injection (red eyes)
 - dry mouth
 - increased appetite
 - altered mental state, short term memory loss and time perception
 - disorientation, disinhibition, agitation, paranoia, visual or auditory hallucinations, anxiety and panic attacks
 - impaired coordination, cognition and judgement
 - seizures
 - hyperemesis

7.3 Assessment and management of intoxication

- The patient may be intoxicated on presentation and this may affect their ability to provide and receive information. Where possible identify as accurately as possible:
 - method of consumption
 - frequency of use
 - amount of money spent per day on cannabis
 - impact on lifestyle.
- Consider differential diagnosis or co-ingestion of other substances if severe symptoms persist.
- The management of cannabis intoxication involves supportive care and depends on clinical presentation.
- Perform and document vital signs on the appropriate AKG Observation and Response Chart.
- Refer to general nursing care requirements

7.4 Cannabis withdrawal signs and symptoms

- Cannabis withdrawal is not life threatening and can be managed in the outpatient setting.
- Inpatient withdrawal may be considered if:
 - repeated attempts at outpatient withdrawal have been unsuccessful
 - the patient has significant mental health issues e.g. schizophrenia, bipolar
 - there is concomitant problematic polysubstance use.

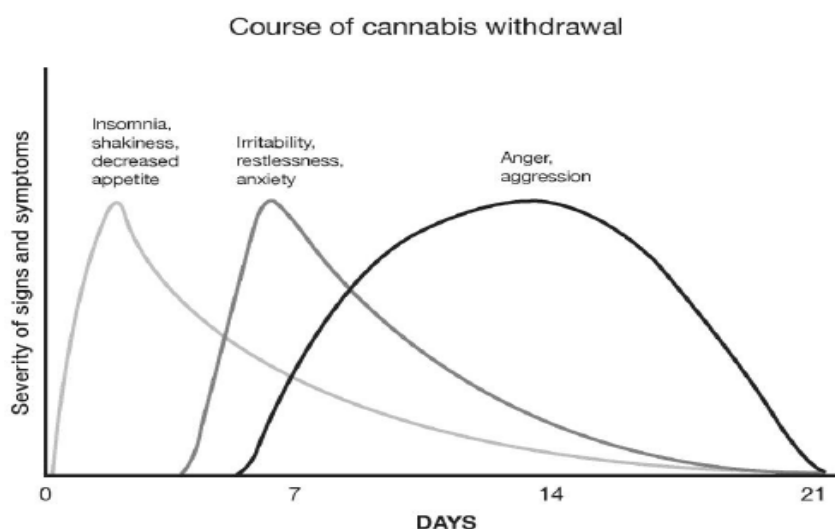
- Symptoms are variable and may not relate to the quantity smoked or duration of use; however, consideration should be given to the following variables which may impact the severity of withdrawal including:
 - psychiatric comorbidity
 - quantity and potency of preparation consumed
 - premorbid tendencies toward aggression and violence
 - duration of current use
 - rate of reduction in use
 - comorbid substance use
 - physical health and current medications.

Clinical studies suggest 50-75% of dependent cannabis users will experience four or more withdrawal symptoms.

Table 9: Symptoms of cannabis withdrawal

Common symptoms	Less common/ equivocal symptoms
Anger or aggression Decreased appetite or weight loss Irritability Nervousness/ anxiety, agitation, restlessness Nausea and vomiting Disturbed sleep and vivid dreams Cravings	Chills Depressed mood Gastrointestinal tract symptoms e.g. abdominal cramps Shakiness, tremor Diaphoresis (sweating), night sweats Hiccups Stuffy nose Fatigue Yawning

Table 10: Course of Cannabis withdrawal



7.5 Most withdrawal symptoms:

- Emerge one to two days after last use

- Peak within the first seven days
- Continue for approximately seven to 14 days.
- Different symptoms may peak at different times throughout the withdrawal e.g. anger and aggression may have a later onset compared to other symptoms.
- Cannabis withdrawal scales are not used at AKGH as there is little research to support their validity, reliability and predictive value
- Cannabis withdrawal syndrome may last for several weeks and patients should be referred to a drug and alcohol service for ongoing support.
- Cannabis withdrawal management in the inpatient setting
- For cannabis withdrawal in the inpatient setting refer the patient to the appropriate AOD service or contact the AOD Clinical Advisory Service as required.
- Patients should be involved in decisions and treatment planning regarding interventions around their cannabis use and be provided with accurate information and harm minimisation advice.
- Supportive therapies for psychological withdrawal may include counselling, motivational enhancement therapy, relapse prevention, cognitive behavioural therapies other psychosocial interventions and self –help groups.
- Two cannabis specific reduction strategies are:
 - sudden cessation, with or without symptomatic relief
 - gradual dose reduction through the use of diaries in combination with evidence based psychological intervention.
- Many cannabis users will be able to stop cannabis use abruptly without experiencing any significant physical withdrawal symptoms.
- Provide symptomatic relief as required, most clinical practice involves the use of benzodiazepines.

Table 11: Pharmaceutical treatment for symptomatic relief

Cannabis Withdrawal Symptoms	Suggested pharmaceutical treatments for symptomatic relief
Nausea & vomiting	Ondansetron sublingual/IV 4-8mg TDS PRN. Metoclopramide PO/IM/IV 5-10mg TDS/PRN Maximum five days of treatment Prochlorperazine PO 5mg, every 4-6 hours as required
Muscle aches & pains and headaches	Paracetamol 1g QID PRN PO
Agitation and/or anxiety, restlessness, irritability	Diazepam (adjust dose to clinical need and risks) PO 5mg up to QID PRN Acute short term use only, up to seven days MH patients refer to the FSFHG Agitation and arousal Mental Health inpatients medication management guideline
Insomnia	Temazepam 10mg nocte PRN PO Maximum of seven days treatment

Cannabis Withdrawal Symptoms	Suggested pharmaceutical treatments for symptomatic relief
Abdominal cramps	Hyoscine butyl bromide 10-20mg TDS PRN PO/IV
Symptoms of psychosis e.g. thought disorders, paranoia, perceptual disturbances	Consider the use of a benzodiazepine Consider oral antipsychotic medications to manage psychotic features as clinically

7.6 Cannabis Hyperemesis Syndrome (CHS)

- Signs and symptoms of CHS include:
 - frequent requests for hot showers
 - severe nausea and vomiting
 - abdominal pain with negative medical workup
 - dehydration and pre-renal acute kidney failure
- Management includes:
 - cessation of cannabis is the only definitive treatment for CHS
 - supportive treatment including fluid resuscitation for dehydration
 - antiemetic e.g. Metoclopramide or Ondansetron if required

8. Opioids

The generic term applied to alkaloids from the opium poppy, their synthetic analogues, and similar compounds synthesised within the body. Includes the illegal drug heroin, synthetic opioids such as fentanyl and pain relievers available legally by prescription such as oxycodone, hydrocodone, codeine, morphine and many others. An overdose occurs when larger quantities than physically tolerated are taken, resulting in CNS and respiratory depression, miosis (pupillary constriction), and apnoea. Overdoses can be fatal if not rapidly treated

8.1 Use of opioids

- Prolonged use of opioids can lead to the development of tolerance, dependence and addiction
- Cessation after prolonged use is associated with a range of withdrawal symptoms. Withdrawal syndrome is uncomfortable but not life threatening.
- Tolerance to opioids decreases rapidly following withdrawal which increases the risk of opioid overdose if the client resumes prior levels of opioid use.
- Harm minimisation advice and ongoing support are crucial to reducing the likelihood of early relapse with potentially fatal consequences.
- Patients not ready for complete withdrawal should consider an opioid substitution treatment program.

8.2 Intoxication signs and symptoms.

Opioid intoxication causes central nervous system (CNS) depression and death may result secondary to respiratory failure.

Classic opioid toxidrome includes:

- CNS depression

- respiratory depression
- Miosis (pupil constriction).
- Some opioids may have unexpected toxicity:
 - Methadone use is associated with QT prolongation
 - Dextropropoxyphene use is associated with seizures and ventricular dysrhythmias
 - Pethidine¹⁵/Fentanyl¹⁶ use may cause serotonin syndrome.

8.3 Intoxication management

- Patient care includes:
 - airway, breathing, circulation management refer to [AKG Code Blue Medical Emergency](#) procedures
 - Naloxone is the specific antidote. Titrated doses of intravenous naloxone in line with RPH Bentley group Intravenous Administration Guidelines for commonly used drugs Standard procedures or consult toxicology services

8.4 Opioid withdrawal

- Opioid withdrawal is the physiological response that results from the abrupt cessation or rapid reduction in opioid dose in an opioid dependent person or when the person is administered an opioid antagonist or partial agonist.
- The aim of withdrawal treatment is to support the patient and provide symptomatic relief from discomfort.
- The onset and duration of withdrawal depends on the half-life of the drug being taken.
- The severity and time course of withdrawal is dependent on the opioid used.

Table 12: Opioid withdrawal course

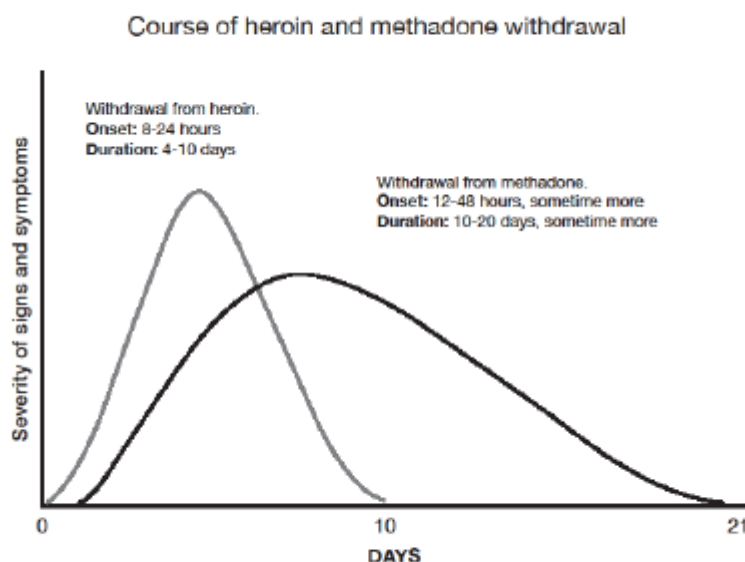


Table 13: Opioid withdrawal symptoms

Stage of withdrawal	Common symptoms
Early	Runny eyes and nose, sneezing, yawning Sweating, hot and cold flushes Loss of appetite Goosebumps
Peak	Strong cravings Stomach cramps and Diarrhoea Nausea and vomiting Body aches Restlessness, agitation, irritability and insomnia Lethargy and poor concentration Hot and cold flushes with increased sweating
Late	Physical symptoms begin to subside Psychological symptoms such as lethargy, irritability, cravings and insomnia may persist but in lower severity

8.5 Clonidine use for management of withdrawal

- Clonidine is accepted for the management of opioid withdrawal symptoms in Australia. It can be used to reduce withdrawal symptoms such as nausea, vomiting, agitation and sweating
- Clonidine has little effect on reducing cravings; however, it does not produce opioid like tolerance or dependence.
- Caution should be used when administering Clonidine with other depressant medications e.g. Benzodiazepines.

Patients taking Clonidine require monitoring for adverse effects including:

- hypotension
- bradycardia
- sedation

Prior to administering first dose of clonidine:

- record baseline blood pressure and heart rate

Clonidine will not be administered if:

- the patient is pregnant
- systolic blood pressure < 90mmHg or
- diastolic blood pressure < 50mmHg or
- heart rate < 50 beats per minute or
- clinical evidence of impaired circulation

Initial clonidine test dose:

- give 75micrograms orally and monitor blood pressure at 30 minutes post administration and document on the Observation and Response Chart lying and standing blood pressure is measured pre and post
- further Clonidine is not recommended if systolic blood pressure is <90mmHg, diastolic blood pressure < 50mmHg, heart rate < 50 beats per minute
- if no adverse side effects e.g. hypotension, dizziness a further 75 micrograms oral dose of clonidine may be administered and continue treatment as per the table of symptomatic treatments below

Table 14: Symptomatic treatment management for mild opioid withdrawal

Symptoms	Suggested treatments
Nausea and vomiting	Ondansetron sublingual/IV 4-8mg TDS PRN. Metoclopramide PO/IM/IV 5-10mg TDS/PRN Maximum 5 days of treatment Prochlorperazine PO 5mg, every 4-6 hours PRN
Muscle aches and Pain	Paracetamol PO 500-1000mg every 4-6 hours PRN. Maximum 4000mg Paracetamol in 24 hours Ibuprofen PO 400mg every 6-8 hours PRN
Agitation and/or anxiety	Clonidine PO 75microgram every 6 hours Diazepam PO 5mg QID PRN MH only see Agitation, arousal Mental Health inpatients Medication management
Insomnia	Temazepam PO 10-20mg PRN at night
Abdominal cramps	Hyoscine butyl bromide PO/IV 10-20mg every six hours PRN Paracetamol may also provide some relief
Diarrhoea	Loperamide PO PRN 2mg after loose bowel motion. Maximum 16mg in 24 hours

8.6 Pregnancy

- Management of opioid dependence/withdrawal in pregnancy is a highly specialised area. Refer pregnant patients to WANDAS for additional specialist advice and ongoing support.
- Opioid withdrawal during pregnancy is a high risk option as relapse will potentially expose both mother and child to illicit substance use.
- Withdrawal prior to 14 weeks gestation or after 32 weeks gestation is associated with a higher risk of:
 - miscarriage
 - foetal distress, death
 - neonatal withdrawal
 - Opioid substitution treatment (OST) is the preferred management for opioid dependent pregnant women.

- Compared to continued illicit drug use or withdrawal, OST is associated with improved foetal development, infant birth weight, and reduced risk of perinatal and infant mortality.
- Patients who decline an OST program should be encouraged to consider withdrawal in the 2nd trimester.
- Risk vs benefit scenarios should be clearly explained to the patient.
- The use of Buprenorphine only (Subutex®) is safe in all trimesters however the use of Buprenorphine/naloxone (Suboxone®) is not recommended. Both are considered safe in breastfeeding. Refer to the [King Edward Memorial Hospital Clinical Practice Guidelines](#) for more information

9. Psychostimulants (amphetamines, cocaine and ecstasy)

A group of drugs which can stimulate the sympathetic and the central nervous system by increasing the synaptic concentration of excitatory neurotransmitters dopamine, norepinephrine and serotonin.

ATS can increase the release of these neurotransmitters, inhibit their reuptake or do a combination of both to produce a range of stimulatory effects. Refers to a group of drugs which includes:

- Amphetamines
- Methylenedioxymethamphetamine (MDMA)

A range of psychostimulants are also available for the treatment of various conditions. These may be used illicitly including:

- Methylphenidate (Ritalin ® and Concerta ®)
- Phentermine (Duromine ® and Metermine ®)
- Pseudoephedrine (Dimetapp ®, Sudafed ®).

Includes the four types of amphetamines:

- Amphetamine
- Dexamphetamine
- Lisdexamfetamine
- Methamphetamine

Most illicit drugs bought as speed or amphetamines in Australia are likely to be methamphetamine.

9.1 Use of psychostimulants

In general stimulants increase alertness, the sense of wellbeing and the pleasurable reinforcement experienced with many activities.

Both intoxication and long-term use of psychostimulants represent a significant burden on the health care system with presentations relating to medical, social and psychiatric sequelae complicating the management of co-existing pathology.

Table 15: Signs and symptoms of psychostimulant intoxication

System	Symptoms
CNS, neurological and behavioural	overstimulation, restlessness, increased energy, reduced fatigue and insomnia heightened alertness, headache, mydriasis, dizziness and tremors overconfidence, violent, unpredictable or irrational behaviour euphoria pressured speech bruxism (teeth grinding) seizures and coma anorexia (appetite suppression) confusion and psychosis (hallucinations, delusions and paranoia)
Cardiovascular	hypertension palpitations tachycardia, arrhythmia angina, acute coronary syndrome, aortic dissection
Gastrointestinal	nausea and vomiting abdominal cramps, Diarrhoea or constipation
Other	tachypnoea (increased respiration), respiratory failure pyrexia (increased temperature) flushing or pallor diaphoresis (sweating)

9.2 Withdrawal

A thorough assessment will include:

- motivation for change
- pattern and route of use
- polysubstance use
- physical assessment
- mental health assessment

Withdrawal can commence in the ED with identification and risk assessment and continued over the length of the admission.

Withdrawal is not life threatening and it is safe for a patient to withdraw in the community setting. This can be difficult for patients with no social support to do so.

Withdrawal management aims to manage symptoms and engage the patient in relapse prevention.

Patients who present solely to reduce or cease their psychostimulant use will be referred to specialist outpatient AOD services to support cessation and abstinence, inpatient admission is not indicated for these presentations.

Table 16: Psychostimulant withdrawal course

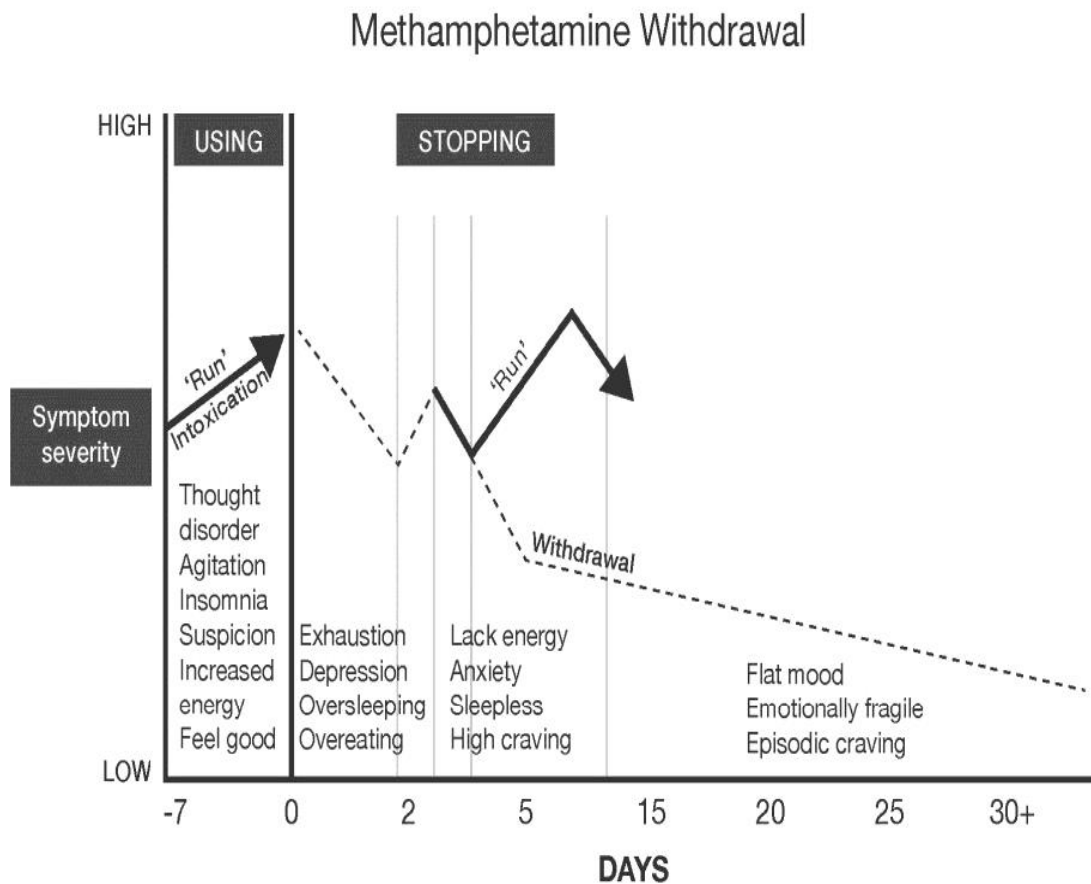


Table 17: Psychostimulant withdrawal

Phase	Time since last stimulant use	Common signs and symptoms
Crash*	Generally, commences 12 to 24hrs after last use and subsides by day 2 to 4. These symptoms are more pronounced following high dose use.	- exhaustion and fatigue - overwhelming desire to sleep or experiencing sleeping disturbances - fluctuating emotional state – dysphoric, and can be associated with anxiety or agitation - low cravings - generalised aches and pains - hunger
Withdrawal*	Generally, commences 2 to 4 days after last use peaks in severity over 7 to 10 days can persist for 6 to 10 weeks	- strong cravings and urge to use - sleeping difficulties and disrupted sleeping patterns - poor concentration and attention - fluctuations in mood and energy levels; alternating between irritability, restlessness,

Phase	Time since last stimulant use	Common signs and symptoms
		anxiety and agitation headaches and generalised aches and pains increased appetite thought disturbance (e.g. predominantly paranoia) and perceptual disturbance (e.g. hallucinations) which may have been masked in the crash phase

9.3 Withdrawal management

- Management of acute presentations includes:
- identifying and treating symptoms/complications of psychostimulant misuse
- providing a safe patient environment
- patient observation and behavioural management
- medical and psychiatric assessment as required
- engagement with AOD services
- for Mental health patients refer to the Agitation, arousal mental health inpatients medication management guideline
- Consider Sympathomimetic toxidrome management.
- Refer to general nursing care requirements

9.4 Psychostimulant use in pregnancy

- Psychostimulant use during pregnancy is associated with harms to the mother and neonate including an increased risk of impaired neonatal growth, preterm labour and delivery. Psychostimulant use can affect the overall development of the baby.
- Hypertension resulting from psychostimulant use can increase the risk of antepartum haemorrhage, cardiovascular accident or acute coronary syndrome
- Withdrawal from ATS during pregnancy is not dangerous – any medication prescribed should be considered in terms of benefit to mother and minimising the risk to the pregnancy.
- Referral to the WANDAS for specialist advice and ongoing support should be discussed and encouraged with the patient to ensure best care for both mother and baby.

10. Volatile Substances (Solvents)

10.1 Intoxication

Volatile substance use (VSU) refers to the practice of deliberately inhaling substances that are vaporous at ambient temperatures, for the purpose of becoming intoxicated. Substances used for this purpose include various hydrocarbons, ethers, ketones and alkyl halides, which are common ingredients in household products.

Intoxication usually occurs within one to five minutes after initial inhalation and can last for several minutes after the person stops inhaling. However, if the person continues inhaling, the effects could potentially last for hours even after the person ceases to inhale.

10.2 Intoxication Signs and Symptoms:

- Euphoria/excitability
- Reduced inhibitions
- Drowsiness/sedation
- Dizziness and headaches
- Impaired judgement, attention and memory
- Blurred vision and slurred speech
- Poor coordination, muscle weakness/lethargy
- Nausea/vomiting
- CNS depression – slowed breathing and heart rate
- Agitation/aggression
- Disorientation/confusion/hallucinations/delusions/delirium
- Decreased level of consciousness
- Cardiac arrest (secondary to catecholamine sensitivity)
- Renal tubular acidosis with symptomatic hypokalaemia (with chronic use)

10.3 Intoxication risks:

Acute risks include:

- sudden death due to hypoxia,
- cardiac sensitisation
- dysrhythmia,
- micro aspiration and pneumonitis

Long term risks include:

- neuropsychiatric impairment,
- renal tubular acidosis from toluene,
- subacute combined degeneration of spinal cord from vitamin B12 deficiency due to nitrous abuse

10.4 Management

Most cases of acute intoxication due to VSU can be managed by removing the substance and letting the person rest. Sedatives should only be used if necessary for acute behavioural disturbance. Specific antidotes may be necessary to treat methaemoglobinemia caused by nitrite exposure or lead toxicity due to inhalation of leaded gasoline

- Perform and document vital signs as clinically indicated.

10.5 Withdrawal

People who use inhaled volatile substances may experience a withdrawal syndrome. However, symptoms are generally not severe and last only a few days (two-to-five days in most people and up to a week in heavy chronic users)

Signs and Symptoms

- Anxiety, irritability, restlessness, agitation

- Nausea, vomiting, diarrhea, abdominal pain
- Tremor and perspiration

Withdrawal Management

- Most cases of acute VSU intoxication can be managed by removing the substance and encouraging rest.
- Management requires treatment of symptoms as necessary.
- Sedatives should only be used if required for acute behavioural disturbances.

11. General nursing care

The following general nursing care applies to all substances:

11.1 Mobility

- A falls risk assessment will be completed in line with the [AKG Fall and Fall Injury Prevention and Management policy](#).
- Consider initially restricting the patient to bed rest.
- Ambulate with supervision when assessed as safe to do so.
- Supervise as indicated with activities of daily living.
- Provide assistance if required with showering/hygiene needs.
- Consider referral to physiotherapy for assessment.

11.2 Diet and hydration

- Encourage oral fluids to correct dehydration.
- Consider commencement of fluid balance chart to monitor intake and output.
- Discourage caffeine where possible.
- Encourage small, regular meals if clinically appropriate.

11.3 Skin integrity

- Care is provided in line with the [AKG Pressure Injury Prevention and Management Policy](#).

12. Involving the patient, family and carers

- Provide patients and families with a treatment setting and experience that is as welcoming, accepting, non-judgmental and responsive as possible.
- Involve patients in decision making and treatment planning.
- Engage family members and people who are significant to the patient in the treatment process (with the patients' approval)
- Provide patient/family with relevant information and resources.
- Involve patients in discharge planning and make them fully aware of their options.
- Identify supports that can be called upon by the patient after withdrawal. Inform of options for seeking further assistance in the future.

13. Alcohol and Inpatients:

Refer to [Alcohol, Illicit Substances, Non-Prescribed Substances Management on the Inpatient Wards Procedure](#)

- The use of alcohol by inpatients will not be permitted.
- If there is reasonable concern that an inpatient has consumed alcohol, the inpatient can be requested to submit to a breath and / or blood test.

14. Legislative context

The following documents are required to give effect to this policy:

- *Mental Health Act 2014*
- *Medicines and Poisons Act 2014*
- *Medicines and Poisons Regulations 2016*
- *Occupational Safety and Health Act 1984*
- Health Department of Western Australia (HDWA) [Alcohol and Other Drug Withdrawal Management Policy MP 0062/17](#)
- HDWA [Opioids, Benzodiazepines and Other Schedule 8 Medicines Prescribing Code](#)
- HDWA [Smoke Free WA Health System Policy MP 0158/21](#)
- HDWA [Medicines Handling Policy MP 129/20](#)
- East Metropolitan Health Service (EMHS) [Alcohol and Prohibited Substances Policy EMHS: 112](#)

15. Supporting information

The following documents support the implementation of this policy:

- Alcohol, Smoking and Substances Involvement Screening Tool (ASSIST) AKMR27
- [Step Up Program Prochaska and DiClemente's Stages of Change Model](#)
- Clinical opiate withdrawal scale – COWS AKMR147.7
- Alcohol Withdrawal Chart (CIWA-Ar) for Alcohol Only AKMR147.6
- AKG Audit and DAST-10
- [Pressure Injury Prevention and Management Policy AKG-POL-0375-17](#)
- [Alcohol, Illicit Substances, Non-Prescribed Substances Management on the Inpatient Wards Procedure MEN-PRO-0023-10](#)
- [Fall and Fall Injury Prevention and Management Policy AKG-POL-0539-20](#)
- [Full Neurological Observations Procedure AKG-GUI-0176-11](#)

16. Compliance, monitoring and evaluation

Compliance with this guideline will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS
- Monitoring of consumer feedback data via Datix CFM

Department specific findings are to be reported at the relevant departmental meeting for review and identification of any opportunities for improvement.

Organisation-wide data trends and risk associated with alcohol and other drugs are to be reported to, and monitored by, the Comprehensive Care Committee.

17. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.29 Safe Environment
- Standard 5 – Action 5.3 Partnering with consumers
- Standard 5 – Action 5.10 Screening of risk
- Standard 5 – Action 5.34 Predicting, preventing and managing aggression and violence

18. References

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19. Document owner

Service Director Mental Health

Enquiries relating to this guideline may be directed to:

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Department: Hospital Wide

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20. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V5	03/10/2024	03/10/2027	Scheduled review

21. Authorisation

This guideline has been authorised and issued by the Service Director Mental Health.

Authorised by	Shannon Stuart – A/Service Director Mental Health
Authorisation date	02/10/2024
Published date	03/10/2024

Appendix 1: Standard drinks guide

NUMBER OF STANDARD DRINKS – BEER

								
1.1	0.8	0.6	1.6	1.2	0.9	1.4	1	0.8
285ml	285ml	285ml	425ml	425ml	425ml	375ml	375ml	375ml
Full Strength	Mid Strength	Low Strength	Full Strength	Mid Strength	Low Strength	Full Strength	Mid Strength	Low Strength
4.8% Alc. Vol	3.5% Alc. Vol	2.7% Alc. Vol	4.8% Alc. Vol	3.5% Alc. Vol	2.7% Alc. Vol	4.8% Alc. Vol	3.5% Alc. Vol	2.7% Alc. Vol
								
1.4	1	0.8	34			24		19
375ml	375ml	375ml	24x375ml			24x375ml		24x375ml
Full Strength	Mid Strength	Low Strength	Full Strength			Mid Strength		Low Strength
4.8% Alc. Vol	3.5% Alc. Vol	2.7% Alc. Vol	4.8% Alc. Vol			3.5% Alc. Vol		2.7% Alc. Vol

NUMBER OF STANDARD DRINKS – WINE

						
1.6	1	0.9	1.4	1	1.4	7.5
150ml	100ml	60ml	150ml	100ml	150ml	750ml
Average Restaurant Serving of Red Wine	Standard Serve of Red Wine	Standard Serve of Port	Average Restaurant Serving of White Wine	Standard Serve of White Wine	Average Restaurant Serve of Champagne	Bottle of Champagne
13.5% Alc. Vol	13.5% Alc. Vol	18% Alc. Vol	11.5% Alc. Vol	11.5% Alc. Vol	12% Alc. Vol	12.5% Alc. Vol
						
8	43	21	7.5	39	19.5	20
750ml	4 Litres	2 Litres	750ml	4 Litres	2 Litres	2 Litres
Bottle of Red Wine	Cask Red Wine	Cask Red Wine	Bottle of White Wine	Cask White Wine	Cask White Wine	Cask of Port
13.5% Alc. Vol	13.5% Alc. Vol	13.5% Alc. Vol	12.5% Alc. Vol	12.5% Alc. Vol	12.5% Alc. Vol	17.5% Alc. Vol

NUMBER OF STANDARD DRINKS– SPIRITS							
							
1 30ml High Strength Spirit Nip 40% Alc. Vol	22 700ml High Strength Bottle of Spirits 40% Alc. Vol	1.1 275ml Full Strength RTD* 5% Alc. Vol	1.2 330ml Full Strength RTD* 5% Alc. Vol	2.6 660ml Full Strength RTD* 5% Alc. Vol	1.5 275ml High Strength RTD* 7% Alc. Vol	1.8 330ml High Strength RTD* 7% Alc. Vol	3.6 660ml High Strength RTD* 7% Alc. Vol
							
1 250ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.2 300ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.5 375ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.7 440ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.4 – 1.9 250ml High Strength Pre-mix Spirits 7% – 10% Alc. Vol	1.6 300ml High Strength Pre-mix Spirits 7% Alc. Vol	2.1 375ml High Strength Pre-mix Spirits 7% Alc. Vol	2.4 440ml High Strength Pre-mix Spirits 7% Alc. Vol

Appendix 2: Drug terminology, common street names and approximate prices of illicit substances

Substance	Measure	Street weight	Approx. cost
Methamphetamine: Chalk, crank, crack, ice, meth and speed, gear.	0.1gram 0.7gram 3.5 grams 7grams 28.3grams	One point Street gram 8 ball (1/8 ounce) 1/4 ball Ounce	\$50-80 \$500 \$800-1000 \$1700 \$5000-7000
Heroin: Big H, horse, hell dust, smack.	0.05 gram 0.1 gram 1 gram 28.3 grams	½ point 1 point Fold/Weight Ounce	\$50 \$60-80 \$500-600 \$10000-12000
Cocaine: Blow, coke, crack, rock, snow.	1gramm 28.3 grams	Ounce	\$350-400 \$8000
Ecstasy: MDMA, eckies, pill, molly	Tablet Tablet	Single Bulk sometimes sheet of 100 tablets	\$10-40
LSD: Acid, trips, Lucy.	Tablet Tablet	Single Bulk sometimes sheet of 100 tablets	\$10-40
Cannabis: Marijuana, weed, cone, choof, grass, pot, gunja, joint, hash	1 gram 28.3grams 454 grams	Bag, foil, stick Ounce, bag Pound (16 ounces)	\$25 - 30 \$250- 300 \$2500-5000
Volatile Substances: Petrol, paint thinner, glue, lighter fluid.	Method of use: huffing, sniffing chroming, bagging		Varies as per chemical used

- The approximate cost is highly variable and based on feedback from patients
- The street weight can be interchangeable between substances i.e. an 'eight ball' can be used to describe 3.5gm of heroin or methamphetamine.
- A speedball is a mixture of heroin and amphetamine
- Asking patients what amount in dollars (\$) or weight in grams is a good idea to add some clarity. Some users may not be able to determine the amount in grams but can tell you "I use two points at a time". This means they are using 0.2 grams. Bear in mind that this 200mg might be 75-85% purity or it might be 10-15% purity.

Appendix 3: Drugs with the potential to interact with alcohol

Medication	Type of interaction
Sedative hypnotics Benzodiazepines Barbiturates	Acute alcohol consumption potentiates the central nervous system depressant effects of benzodiazepines and barbiturates. Risk of cognitive impairment, respiratory depression and overdose is increased. Chronic alcohol consumption decreases availability of barbiturates through hepatic enzyme induction, decreasing their effect.
Anaesthetic agents	Chronic alcohol consumption: Increases the dose of propofol required to induce anaesthesia Increases the risk of liver damage by anaesthetic gases enflurane and halothane.
Opioid analgesics	Alcohol increases sedative effect. Risk of cognitive impairment, respiratory depression and opioid overdose
Tricyclic antidepressants	Acute alcohol consumption increases risk of sedation and orthostatic hypotension (sudden drop in blood pressure upon standing up).
Antihistamines	Alcohol potentiates the central nervous system depressant effect of sedating antihistamines, especially in elderly people.
Antipsychotic medication: Phenothiazines	Acute alcohol consumption increases sedative effects, impairs coordination and may result in liver impairment. Alcohol increases sedation and risk of hypotension.
Oral hypoglycaemic agents: Sulfonylurea compounds	Diabetics on sulfonylureas should be advised not to drink. Acute alcohol ingestion prolongs availability of hypoglycaemic agents leading to hypoglycemia. Hypoglycemia may also occur if there is malnutrition or depletion of glycogen stores.
Anticonvulsants: Phenytoin	Acute alcohol consumption increases availability of Phenytoin increasing risk of side effects.
Histamine H2 receptor antagonists:	These drugs inhibit gastric alcohol dehydrogenase and increase the rate of gastric emptying. This may increase blood alcohol concentration.
Oral anticoagulants: Warfarin	Acute alcohol consumption increases Warfarin's availability, increasing risk of haemorrhages. Chronic alcohol consumption reduces availability of Warfarin, decreasing its
Non-narcotic analgesics: Aspirin, NSAIDs	Alcohol increases the risk of gastrointestinal bleeding.
Paracetamol	Chronic alcohol consumption increases risk of liver damage with paracetamol overdose